

Health Regulation & Licensing Administration

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: HFD01-0188	(X2) MULTIPLE CONSTRUCTION A. BUILDING: _____ B. WING _____	(X3) DATE SURVEY COMPLETED C 12/18/2025
NAME OF PROVIDER OR SUPPLIER MEDSTAR GEORGETOWN UNIVERSITY		STREET ADDRESS, CITY, STATE, ZIP CODE 3800 RESERVOIR RD WASHINGTON, DC 20007		
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H 000	Initial Comments An unannounced complaint [DC~00013935 & DC~00012682] survey was conducted at Medstar Georgetown University Hospital from 12/15/2025 to 12/18/2025. The facility's census was 432 on the first day and 518 on the last day of survey. The sample size was 9 patients. Deficiencies were cited related to the investigation of DC~00013935. There were no deficiencies cited related to DC~00012682. The following deficiencies are based on observations, medical record review, hospital policy review, and staff interviews.	H 000		
H 145	2024.4 Hospitals Each hospital shall provide the necessary care and treatment to meet the needs of patients. Care and treatment provided shall meet prevailing professional standards and scope of practice requirements. Each hospital shall establish and implement written policies and procedures that encompass care and treatment provided to patients. This CONDITION is not met as evidenced by: Based on medical record review, policy review, and staff interview, 1) the nursing staff failed to document oxygen therapy administration to include the flow rate as ordered by the physician for one (1) of four (4) patients (Patient #1) and 2) the Respiratory Care staff failed to document the	H 145	H145 Nursing General Surgery The nursing staff failed to document oxygen therapy to include the flow rate as ordered by the physician in 1 out of 4 patients. Plan for correcting the deficiency Unit staff will be re-educated on the expectation that oxygen therapy flow rate must be documented in the medical record as ordered by the physician Plan to improve process(es) to prevent recurrence <i>The unit nurse education shall develop education in the form of SBAR highlighting the expectation of oxygen therapy flow rate documentation.</i> Procedure for implementing the corrective action plan The SBAR will be sent to all unit staff via the unit newsletter and the topic will be reviewed in unit huddles throughout the last 2 weeks of May and the first two weeks of June. Date that corrective action to be completed Corrective action to be completed July 8 2026 How you plan to monitor corrective action for sustainment A report of patients on the unit for whom oxygen therapy was ordered will be developed by Nursing Informatics .	

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		The Nursing Leadership of the unit will audit 5 randomly selected records per week. A goal of 90% compliance for 3 consecutive months shall be established. Results of audits shall be reviewed at the monthly Nursing Executive Council meeting. Title of the person responsible for implementing the plan of correction The Senior Director of Surgical Nursing with direct oversight for this unit will be responsible for implementing the plan of correction.	
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LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

● *Phyllis Morris-Griffith PhD RN*

TITLE
AVP Regulatory Readiness (X6) DATE
05/28/2026

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H 145	Continued From page 1 administration and reassessment of ordered Bilevel Positive Airway Pressure (BiPAP) therapy for one (1) of four (4) patients (Patient #1). Findings include: 1. Review of the medical record showed Patient #1 was admitted with a diagnosis to include but limited to COPD, Lung Cancer, Heart Failure, and Brain Aneurysm. Further review of the medical record showed a physician order dated 10/09/2025 for continuous oxygen therapy via nasal canula at 2 liters per minute. Review of nursing documentation indicated that nursing assessments did not consistently include documentation of Patient #1's oxygen flow rate administered at the time routine vital signs were obtained. Specifically, oxygen flow rate documentation was absent: 10/15/2025 at 08:00 AM, 12:00 PM, 4:00 PM, and 8:00 PM. The surveyor conducted a face-to-face interview with Employee #2, Assistant Vice President Regulatory Readiness, on 12/16/2025 at approximately 10:30 AM. The employee confirmed that documentation of the oxygen flow rate was missing at the times identified above. The surveyor conducted a face-to-face interview with Employee #6, Assistant Nursing Director on 12/17/2025 at approximately 10:40 AM. The employee stated that nursing staff are expected to consistently document the oxygen delivery device and oxygen flow rate at the time vital signs are obtained. 2. Review of the facility's policy titled "	H 145	H145 Respiratory Therapy (C62) The respiratory therapy staff failed to document admin Plan for correcting the deficiency The Respiratory Therapy Staff assigned to this unit will be positive pressure ventilation. Plan to improve process(es) to prevent recurrence All Respiratory therapy staff assigned to this unit will receive <u>Ventilation</u> that outlines the expectations for <u>documentat</u> (BiPAP) along with a message from the Director outlining policy and initial receipt. The documentation requirements Procedure for implementing the corrective action plan The policy will be sent to all applicable respiratory therapy Date that corrective action to be completed Corrective action to be completed July 8, 2026 How you plan to monitor corrective action for sustainm A report of patients on the unit for whom BiPap was ordere audit 10 randomly selected records per week. A goal of 90% compliance for 3 consecutive months shall be Council meeting. Title of the person responsible for implementing the pla The Director of respiratory Therapy will be responsible for	

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H 145	Continued From page 2 Non-Invasive Positive Pressure Ventilation" revised 3/1/2024 showed "Procedure: a) The Respiratory Therapist will initiate and make changes as ordered by the physician ...c) Assessment will be documented in the patient's electronic medical record. Monitoring: a) NIPPV is assessed and documented Q [every] 4hours ..." Review of the medical record showed Patient #1 was admitted with a diagnosis to include but limited to COPD, Lung Cancer, Heart Failure, and Brain Aneurysm. Further review of the medical record showed a physician order dated 10/13/2025 nocturnal Bi-Level Positive Airway Pressure (BiPAP) therapy (Non-Invasive). Review of documentation for the night of 10/16/2025 showed no evidence that Patient #1 received BiPAP therapy as ordered. Review of documentation dated 10/14/2025 showed Patient #1 received BiPAP therapy at 12:45 AM; however, required every-four-hour reassessment documentation was not completed. The next documented assessment occurred at 9:41 AM on 10/14/2025, approximately nine (9) hours later. The surveyor conducted a face-to-face interview with Employee #5, Respiratory Therapist Director on 12/16/2025 at approximately 3:15 PM. According to the employee, Respiratory Therapists are responsible for documenting when BiPAP therapy is administered in accordance with the physician's orders. The employee further stated that if BiPAP therapy is not administered as ordered, the Respiratory Therapist is expected to document the reason. Additionally, the	H 145		

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H 145	Continued From page 3 employee stated that reassessments should be completed and documented every four hours while the patient is receiving BiPAP therapy. The employee confirmed the missing documentation identified above.	H 145		
H 146	2024.5 Hospitals A plan of care shall be established, implemented and kept current to meet the identified needs for each inpatient. The plan of care shall be interdisciplinary when appropriate to meet individual needs of patients. This CONDITION is not met as evidenced by: Based on medical record review and staff interview, nursing staff failed to document a care plan to address the patient's individualized needs for one (1) of four (4) patients (Patient #1). Findings include: Review of the medical record showed Patient #1 was admitted with a diagnosis to include but limited to COPD, Lung Cancer, Heart Failure, and Brain Aneurysm. Further review of the medical record showed a physician order dated 10/09/2025 for continuous oxygen therapy via nasal canula at 2 liters per minute and a physician order dated 10/13/2025 for nocturnal Bi-Level Positive Airway Pressure (BiPAP) therapy (Non-Invasive).	H 146	H146 Nursing General Surgery The nursing staff failed to document a care plan to address the patient's individualized needs. Plan for correcting the deficiency Unit staff will be re-educated on the expectation that plans of care be Individualized. Plan to improve process(es) to prevent recurrence <i>The unit nurse education will develop education for the unit staff on the clinical parameters for implementing the IPOC as well as initiating the IPOC when it is autosuggested in the electronic medical record based on key documentations elements entered.</i> Procedure for implementing the corrective action plan <i>The education will be sent to all unit staff via the unit newsletter and the topic will be reviewed in unit huddles throughout the last 2 weeks of May and the first two weeks of June.</i> Date that corrective action to be completed Corrective action to be completed July 8, 2026 How you plan to monitor corrective action for sustainment A report of patients on the unit for whom the specific IPOC was suggested will be developed to assist with ongoing monitoring. Nursing Leadership will review 10 random records a week with compliance calculated via the number	

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			of suggested IPOC topics and the actual initiation of the suggested IPOC. A goal of 90% compliance for 3 consecutive months shall be established. Results of audits shall be reviewed at the monthly Nursing Executive Council meeting. Title of the person responsible for implementing the plan of correction The Senior Director of Surgical Nursing with direct oversight for this unit will be responsible for implementing the plan of correction.	
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H 146	Continued From page 4 Review of the patient's care plan showed a lack of documented interventions to address Patient #1's respiratory needs. The surveyor conducted a face to face interview with Employee #6, Assistant Vice President Regulatory Readiness, on 12/18/2025 at approximately 1:30 PM. The employee confirmed that Patient #1's care plan referred to as the Interdisciplinary Plan of Care (IPOC) should have included interventions to meet the patient's respiratory needs, however such interventions were not documented.	H 146		
H 172	2030.3 Hospitals Each medical record shall contain, when applicable, the following information: (a) Identification data; (b) Chief complaint; (c) Present illness; (d) History and physical examination; (e) Admitting diagnosis; (f) All pathology/laboratory and radiology reports; (g) Properly executed informed consent forms; (h) Consultation reports; (i) Medical practitioner orders;	H 172		

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H 172	Continued From page 5 (j) Documentation of all care and treatment, medical and surgical; (k) Tissue report; (l) Progress notes of all disciplines; (m) Discharge summary and final diagnosis; (n) Autopsy findings; and (o) Advanced directives, if available. This CONDITION is not met as evidenced by: Based on medical record review, policy review and staff interview, the case management and social worker staff failed to complete an initial assessment within the required time frame for one (1) of four (4) patients (Patient #1). Findings include: Review of the facility's policy titled "Inpatient Charting" revised 03/2023, showed "Initial Assessment; Social Workers: Documents results of all consults within two business days of receipt of the consult." Review of the medical record showed Patient #1	H 172	172 Hospital Case Management The Case Management and Social Worker failed to complete an initial assessment within the time frame for 1 of 4 patients. Plan for correcting the deficiency <i>All Case Managers will be re-educated about the facility's Inpatient Charting Policy, specifically expectations for completion and documentation of initial case management assessments within 48 hours of consult order entry and the escalation expectations when consult barriers are encountered.</i> A root cause analysis determined this case was related to a weekend and multiple unit transfers and a breakdown in consult tracking, and workflow oversight. The initial assessment and consult follow up was completed late. Immediate corrective action included review of the cited patient record and reinforcement of expectations with case management staff regarding required completion of initial assessments within 2 business days of consult order receipt. Plan to improve process(es) to prevent recurrence <i>The Director, Case Management will create an SBAR for case managers on the expectation for completion and documentation of the initial case management assessment within 48 hours of consult order entry.</i> <i>In addition, the policy will be revised to reflect that the consult be completed within 2 business days rather than 48</i>	

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			hours. Procedure for implementing the corrective action plan The education will be sent to all case managers and the topic will be reviewed in upcoming staff meetings throughout the last 2 weeks of May and the first two weeks of June. Date that corrective action to be completed Corrective action to be completed July 8, 2026 How you plan to monitor corrective action for sustainment A report of initial case management consult orders will be developed to assist with ongoing monitoring. Case Management leadership will audit 10 random records a week with compliance calculated as the number of initial case management consults ordered and the number of initial consults completed within 2 business days. A goal of 90% compliance for 3 consecutive months shall be established. Results of audits shall be reviewed at the monthly Quality and Safety Executive Council. Title of the person responsible for implementing the plan of correction The Director, Case Management will be responsible for implementing the plan of correction.	
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H 172	Continued From page 6 was admitted with a diagnosis to include but limited to COPD, Lung Cancer, Heart Failure, and Brain Aneurysm. Further review of the medical record showed a physician order dated 10/09/2025 for a case management consult due to Patient #1's lack of a support system at home. Review of the medical record revealed that a case management initial assessment was not completed in accordance with the physician's order and facility's policy. The surveyor conducted a face to face interview with Employee #2, Assistant Vice President Regulatory Readiness, on 12/15/2025 at approximately 2:30PM. According to the employee case management initial assessments are expected to be completed and documented within forty-eight (48) hours of the consult order. The employee further confirmed that the initial assessment for Patient #1 was not documented.	H 172		